



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : صباح وهيب يوسف مصطفى

العنوان : الاقواز

الحى / المدينة : الصف

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 30-09-2026

رقم البطاقة : 26312172100589 رقم التليفون : 01126755374

العمر (تاريخ الميلاد) : 17-12-1963 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/ د/ د/ د/

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: صباح وهيب يوسف مصطفى National ID: 26312172100589	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM

22/ 6 / 2025 G

Name	صباح وحيث يوسف	MRN	3933
Age	61 y.	ID	2024
Menopausal status	Menopausal	Phone	01126755374
NRS SCORE		Marital status	M.
BMI		Family history	irrelevant
CO morbidity	irrelevant	REASON OF MDT	Post-op.

Clinical Data

BX: I.D.C	R			
ER: 7/8	PR: 3/8		HER 2: 3	Ki67: 25%
Sonomammography:				
Mets Workup: 2t MRM (post NAC) No residual viable tumor detected				

Recommendations:

Free deep margin
Free nipple and areola.
Free L.Ns (0/14)
pathologic complete response,

MDT OUTCOME & PLAN

Sub A just phesgo + ports + famer.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: صباح وهيب يوسف MR. : 1733	
Detailed Medical Report	
السيد	
تحية طيبة وبعد,,	
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته	
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه The multidisciplinary team (MDT) documented that the patient had completed neoadjuvant chemotherapy with a complete pathological response. The treatment plan at that time included continuation with PHESGO and FEMARA, and the patient was noted to be on cycle 15 (C15) of PHESGO + FEMARA. The subsequent note indicated that the patient progressed to cycle 16 (C16) of PHESGO + FEMARA, with an echocardiogram performed on the same day, and then to cycle 17 (C17) of PHESGO, with pharmacy coordination noted. During a clinical pharmacy visit, the patient was recorded as receiving PHESGO + FEMARA in cycle 17 (C17) with an estimated efficacy (EF) of 63 % as of 2 March 2026. The oncology medical note reported that the patient complained of swelling in the mouth. On examination, there was swelling of the floor of the mouth at the site of the sub-mandibular duct, described as "mostly for us." On the most recent examination, a lesion was identified on the floor of the mouth, and a biopsy was planned along with a neck ultrasound for further evaluation. The current plan is to obtain tissue diagnosis of the oral lesion and assess cervical lymph nodes with ultrasound.	
تحريراً في ,	
التاريخ :	20-09-2026
الطبيب المعالج :	نهى سعيد علي محمد
يعتمد ,	



Patient Name : الأستاذ / صباح وهيب يوسف مصطفى	Request No. : 13009827
Patient No. : 26312172100589	Request Date : 19/10/2024
Path No. : 3998/24	Verification Date : 31/10/2024
Sex : Female	Financial Category : أورام دار السلام - مبادئه
Age : 61 Y	Department : Out Patient
Referreing Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left retro-areolar breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 17/10/2024.

Gross Examination:

- Three grayish white tissue cores up to 2 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Not detected.
- * Perineural invasion: Not detected.
- * Angioinvasion: Not detected.
- * Microcalcifications: Not detected.

Diagnosis:

Left retro-areolar breast mass., Tru-cut biopsy:

Invasive duct carcinoma., Grade II.

For further IHC (ER, PR, Her2, Ki67).

Signature
Dr.Salma Tark
Dr.Menna Nabil

Verified by :

Printing Date : 10/11/2024 10:38:42

Page 1 of 1

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / صباح وهيب يوسف مصطفى
 Patient No. : 26312172100589
 Path No. : 1817/25
 Sex : Female
 Age : 61 Y
 Referring Doc :

Request No. : 13005889
 Request Date : 18/05/2025
 Verification Date : 03/06/2025
 Financial Category : اورام دار السلام - مبادره
 Department : Out Patient
 Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer (post NACT)
 Nature of the specimen: Left MRM

Gross Examination:

• Procedure: Left MRM

measured 24x20x7 cm with covering skin ellipse measured 22x20cm & attached axilla 9x6 cm.

• Tumor extent : A greyish white tumour bed site

• Size: 4x3x3 cm.

• Skin Involvement: Nipple & areola were unremarkable grossly.

• Distance from margins: 2 cm away from deep margin

• Lymph nodes number and size: 14 L.Ns up to 2x1 cm.

Endoscopic examination:

• Histological type: No residual viable tumor tissue detected

• Ductal carcinoma in situ: No

• Skin Involvement: Free nipple and areola

• Lymph node involvement: Free L.Ns (0/14)

• Resection Margins: Free deep margin

• Additional pathological findings: Positive microcalcification

• Histologic response to therapy: Pathologic Complete response

Diagnosis:

Left MRM (post NACT) :

No residual viable tumor tissue detected.

• Free deep margin

• Free nipple and areola

• Free L.Ns (0/14)

Pathologic Complete Response

Stages : ypT0 , ypN0

Verified by :



Patient Name : الأستاذ / صباح وهيب يوسف مصطفى
Patient No. : 26312172100589
Path No. : 3998/24
Sex : Female
Age : 61 Y
Referring Doc. :

Request No. : 13009827
Request Date : 19/10/2024
Verification Date : 09/11/2024
Financial Category : اورام دار السلام - مبادره
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Left breast retroareolar mass.

Nature of Specimen:

Tru-cut biopsy.

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

IPT (Positive control showed appropriate reactivity):
ER: Moderate nuclear reaction in 40% of tumor cells (score 7/8).
PR: Moderate nuclear reaction in 10% of tumor cells (score 3/8).
Her2: Complete membrane reaction in >10% of tumor cells (score 3+).
Ki67: 25% nuclear reaction of tumor cells.

Conclusion:

Left breast retroareolar mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.

DT:

ER: Positive (score 7/8).
PR: Positive (score 3/8).
Her2: Positive (score 3+).
Ki67: Positive in 25% of tumor cells.

Verified by :

Printing Date : 25/12/2024 11:37:34

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	الأستاذة / صباح وهيب يوسف مصطفى	Request No.	13009827
Patient No.	26312172100589	Request Date	19/10/2024
Path No.	3998/24	Verification Date	09/11/2024
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	61 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, 1-10%; 2, between 11% and 33%; 3, between 34% and 66% and 4, between 67% and 100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.
Scoring system: "Her2"
ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Salma Tarek
Esraa Magdy

Verified by :

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Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Sabah Wahib Youssef Moustafa	Date: 15/10/2024
Exam: MAMMOGRAPHY BOTH	Age: 61 Year(s)
Medical ID: 26312172100589	Pat. ID: 90734

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Right breast palpable mass

Report:

- Left Breast:
 - Partially obscured high density spiculated mass is seen in the retro-areola region invading the nipple and ulcerating over the skin. It measures about 45x35mm. by U/S it is seen as a large irregular hypoechoic mass centered at 9 o'clock (extending into the surrounding tissue with intraductal extension reaching the nipple)
 - Another small suspicious mass is noted at 8O'clock measuring about 15x13mm
 - <3 Pathologically enlarged level I axillary LNs largest measures 15mm in TS diameter and 12mm cortical thickness.
 - No pathological enlarged infra or supraclavicular lymph nodes.
- Right Breast:
 - Scattered fibroglandular parenchyma (ACR b)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Left locally advanced breast malignant mass with ipsilateral axillary lymphadenopathy (BIRADS 5) for histopathological examination.
- Normal sonomammography of right breast (BIRADS 1)

Best regards,
Dr Eman Ebeid
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
15/10/2024 3:33 PM



Patient Name: Sabah Wahib Youssef Moustafa	Date : 27/01/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 61 Year(s)
Medical ID: 3933/2024	Pat. ID: 90734

BILATERAL BREASTS ULTRASOUND

Indication:

Left breast malignant mass on NACTH

Report:

- Left Breast:
 - Mild regressive course of the large irregular hypoechoic mass centered at 9 to 10 o'clock (extending into the surrounding tissue with intraductal extension reaching the nipple) measuring currently 23 X 49 mm.
 - Currently noted 8 to 9 o'clock area of altered parenchymal with intraductal tumoral extension.
 - Regression of the <3 Pathologically enlarged level I axillary LNs largest measures 11mm in TS diameter and 6 mm cortical thickness.
 - No pathological enlarged infra or supraclavicular lymph nodes.
- Right Breast:
 - Scattered fibroglandular parenchyma (ACR b)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Regressive course of the left locally advanced breast malignant mass and ipsilateral axillary lymphadenopathy (BIRADS 6).
- Normal sonomammography of right breast (BIRADS 1).

Dr. Mohamed Mohsen, M.D.



Patient Name: Sabah Wahib Youssef Moustafa	Date : 02/06/2025
Exam : MAMMOGRAPHY BOTH	Age : 61 Year(s)
Medical ID: 3933/2024	Pat. ID: 90734

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Left mastectomy

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

Right breast:

- The right breast is predominantly fatty (ACR A)
- No spiculated masses or grouped microcalcifications.
- Normal skin thickness and nipple-areola complex.
- Nonspecific right axillary node, with preserved fatty hila.

Left operative bed :

- The operative bed exhibits an underlying cystic collection measuring about 7x1.5cm, along with another seen in the axillary region, measuring about 6.5x1.5cm.

No definite pathological axillary node is noted.

Opinion:

- Left MRM with postoperative seromas
- Right normal study (BIRADS 1)

Best regards,
Dr. Mahi Mokhtar



Patient Name: Sabah Wahib Youssef Moustafa	Date : 02/06/2025
Exam : MAMMOGRAPHY BOTH	Age : 61 Year(s)
Medical ID: 3933/2024	Pat. ID: 90734

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Left mastectomy

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

Right breast:

- The right breast is predominantly fatty (ACR A)
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Left operative bed :

- The operative bed exhibits an underlying cystic collection measuring about 7x1.5cm, along with another seen in the axillary region, measuring about 6.5x1.5cm.
- No definite pathological axillary node is noted.

Opinion:

- Left MRM with postoperative seromas
- Right normal study (BIRADS 1)

Best regards,
Dr. Mahi Mokhtar

WAHIB, SABAH Patient ID: 10110324016451 DOB: 17-Dec-1963 Date: 01-Jan-2025

Patient Lastname: Wahib

Patient Firstname: Sabah

HIS Patient ID: 10110324016451

RIS Patient ID: 363996

Date of Birth: 17/12/1963

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (1/1/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

" No definite evidence of active osseous deposits could be seen over the surveyed skeleton.

" Degenerative changes are noted over the dorso-lumbar vertebrae.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mahmoud Akram

Mohamed Abdeltawab, MSc

===== REPORT TEXT ENDE =====

Q4
9-7

Date Of Submission:

09/02/2026
13:43:37

Date Of MDT:

09/06/2026 11:47:00

General MDT form

Patient Name:	Salah Wahib Yousef //	Age:	63 Y
Sex:	Female	Special Habits:	26312172100589
ID:		Medical ID:	1733
Family History:		Comorbidity:	FREE
Surgical History:		Telephone:	01126755374
Menopausal Status:		Marital Status:	
Referring Physician:	MDT	Reason Of MDT:	NEW
MDT Name:	Thoracic		

Medical Report

Diagnosis:	
Pathology:	by exam there is rt oral cavity mass at 2nd molar ABOUT 1 X 1 CM
IHC:	
T:	
N:	
M:	
History:	
Radiology Investigation:	
Recommendations :	ORAL CAVITY MASS FOR EXCISIONAL BIOPSY
MDT DECISION:	ORAL CAVITY MASS FOR EXCISIONAL BIOPSY for H&E

Signature	Medical Oncology	Surgery	Radiology	Pathology	Nutrition	Psychiatry
	د. طارق عزالدين	د. محمد العبدالله				

٣٦٣٣
٢٢

83242785
صاح وثيقة هوية
الرقم
التاريخ
البلد
HOS.NO:

جمهورية مصر العربية
بطاقة تحقيق الشخصية



صباح
وهيب يوسف مصطفى
قرية الاقواز
مركز الصف - الجيزة

١٩٦٣/١٢/١٧ ٢٦٣١٢١٧٢١٠٠٥٨٩
JK7416201

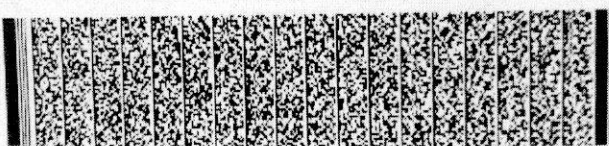


مباركة

مستطبة
١٩٢٠

٢٠٢٢/٠٦ ٢٦٣١٢١٧٢١٠٠٥٨٩
بدون عمل
أنثى مسلمة أرملة

البطاقة سارية حتى ٢٠٢٩/٠٦/١٧



١٩٦٧٠٥٠٧٧٩



Patient No. : 3933-2024
Patient Name : الأستاذ / صباح وهيب يوسف مصطفى
Sex : Female Age: 61 Y
Financial Cate : أورام دار السلام - مبدرة
Department : طوارئ
Referring Doc :

Request No : 13005123
Request Date : 15-05-2025 10:41
Verification Date : 15-05-2025 14:05
Sample Type : Serum
Test Status : Routine
Analyzers : Mini_Vidas_Hermel

Virology Report

Test	Result	Ref. Range
HBs Ag	NEGATIVE	
HCV_Ab	2.91	Negative 0 - 1 Positive greater than 1
HIV Ab	NEGATIVE	

Signature

Dr.Reaam Abd El Menaem



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : صباح وهيب يوسف مصطفى

Sex:

3- Medical number:

Female

83242785

Old Med :

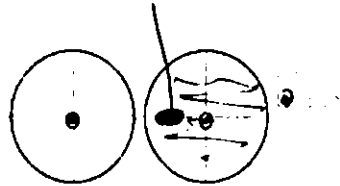
Old Med :

3933-2024

2-ID number:

Patient Age : 61 Y

gms/steer



Family history	Family History	Phone	Phone	BMI	BMI	REASON OF MDT	REASON OF MDT
			01126755374				N/A
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT
					C		09/12/2024
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE		
	CHOLE CYSTECTOMY		POSTMENOPAUSAL				

Clinical data

Breast Imaging : It locally advanced breast malignant mass with ipsilateral axillary lymphadenopathy BIRADS 5			Mets Workup : CTCAP FREE		
BX IDC	ER 7/8	PR 3/8	Her 2 Neu 3	SISH	KI67 25
Bx COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT	

T CT4B	N 1	M 0 BY CT
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Sonomammography

PATIENT HISTORY
ECHO, LABS
BONE SCAN

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

NACT TAXO CARBO WEEKLYX18 PHESGOX6 FOLLOWED BY LT MASTECTOMY

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/الاء جميل	RADIOLOGY	RADIOLOGY د/مروة حكيم
SURGERY	SURGERY د/محمود علي	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



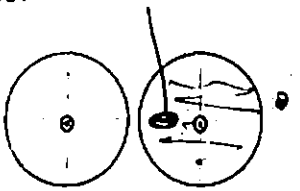
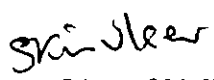
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البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:
1- Name : صباح وهيب يوسف مصطفى
2-ID number:
3- Medical number: 83242785
Female
Old Med : 3933-2024
Old Med : 3933-2024
Patient Age : 61 Y



Family history
marital status
Co morbidity

Family history
Marital Status
Co Morbidity

Phone
NRS Score
Menopausal status

Phone
NRS Score
Menopausal Status

BMI
GROUP
Initial STAGE

BMI
GROUP C
Initial STAGE

REASON OF MDT
Date of MDT

REASON OF MDT
Date of MDT

CHOLE
CYSTECTOMY

POSTMENOPAUSAL

09/12/2024

Clinical data

Breast Imaging :
It locally advanced breast malignant mass with ipsilateral axillary lymphadenopathy BIRADS 5

Mets Workup :
CTCAP FREE

BX
IDC
BX COMMENT:

ER
7/8
ER COMMENT:

PR
3/8
PR COMMENT:

Her 2 Neu
3
Her 2 Neu COMMENT:

SISH
SISH COMMENT:

KI67
25

T
CT4B

N
1

M
0 BY CT

Sonomammography

PATIENT HISTORY
ECHO,LABS
BONE SCAN

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
NACT TAXO CARBO WEEKLYX18 PHESGOX6 FOLLOWED BY LT MASTECTOMY

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/الاء جعين	RADIOLOGY	RADIOLOGY د/مروة حليم
SURGERY	SURGERY د/محمود طاهر	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

MDT FORM

/ / 20

G

Name	م. ح. حبيب	MRN	٢٩٢٢
Age	٦٥	ID	٢٤
Menopausal status	post	Phone	٨٣٩٤٩٧٨٥
NRS SCORE		Marital status	- ١١٩٦٧٥٥٢٧
BMI		Family history	- ١٦٤٢١٧٧٦٢٧
CO morbidity		REASON OF MDT	

Clinical Data

BX:	R	L
ER: 7/5	PR: 7/5	HER 2: +3
Sonomammography:		
Mets Workup:		
TNM T1 N0 M0		

Recommendations: 1st LABC, received NAIT
Tct/A x 18 mks, with good clinical
response for surgery.

MDT OUTCOME & PLAN
for left MRM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT FORM 22/ 6 / 20 25 G

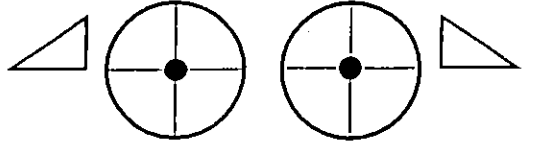
Name	صبا 2 و صبا يوسف	MRN	3933
Age	61 y	ID	2024
Menopausal status	Menopausal	Phone	01126755374
NRS SCORE		Marital status	M
BMI		Family history	inherent
CO morbidity	inherent	REASON OF MDT	post-op

Clinical Data

BX: I.D.C			
ER: 718	PR: 318	HER 2: 3	Ki67: 25%
Sonomammography:			
Mets Workup:			
2t MRM (post NAC) No residual viable tumor detected Free deep margin			

R L

Wound healing



TNM
4B10
4th-NACT

Recommendations:

Free nipple and areola.
Free L.Ns (0/14)
pathologic complete response.

MDT OUTCOME & PLAN

Sub A Joint phesgo + ports + femoral

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : صباح وهيب يوسف مصطفى

Sex:

3- Medical number:

Female

83242785

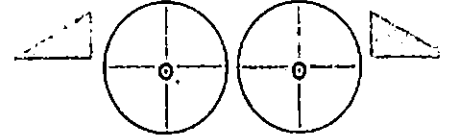
Old Med :

Old Med :

3933-2024

2-ID number: 26312172100589

Patient Age : 61 Y



Family history

Family History

Phone

Phone

01126755374

BMI

AMI

REASON OF MDT

REASON OF MDT
POST NACT

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP
C

Date of MDT

Date of MDT
2025-05-05

Co morbidity

Co Morbidity

Menopausal
status

Menopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE

CHOLE
CYSTECTOMY

Clinical data

Breast Imaging :

It locally advanced breast malignant mass with ipsilateral axillary lymphadenopathy BIRADS 5

Mets Workup :

CTCAP FREE

BX
IDC

BX COMMENT:

ER
7/8

ER COMMENT:

PR
3/8

PR COMMENT:

Her 2 Neu
3

Her 2 Neu COMMENT:

SISH

SISH COMMENT

KI67
25

T
CT4B

N
1

M
0 BY CT

Sonomammography

PATIENT HISTORY

It LABC received NACT TCHP x 18 wks with good clinical response

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

LT MRM POST NACT

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ نورا قاسم د/ نورا قاسم	RADIOLOGY	RADIOLOGY د/ مروة حسن
SURGERY	SURGERY د/ عمرو مظهر	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Patient Name	: الأستاذ / صباح وهيب يوسف مصطفى	Request No.	: 13009827
Patient No.	: 26312172100589	Request Date	: 19/10/2024
Path No.	: 3998/24	Verification Date	: 09/11/2024
Sex	: Female	Financial Category	: أورام دار السلام - مبلّغ
Age	: 61 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: marwa mchamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Left breast retroareolar mass.

Nature of Specimen:

Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 40% of tumor cells (score 7/8).
- PR: Moderate nuclear reaction in 10% of tumor cells (score 3/8).
- Her2: Complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 25% nuclear reaction of tumor cells.

Conclusion:

Left breast retroareolar mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 7/8).
- PR: Positive (score 3/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 25% of tumor cells.

Verified by :





pp

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام

Echo report

صباح و هيب

٢٩٢٢
٢٤

Name:

Age:

Date: ٤/٩/ 2025

EF	62%	54-74%	PWT		0.6-1.1cm
LVEDD	4.9	3.5-5.7cm	AO		2.0-3.7cm
LVESD	3.2	2.2-4.0cm	LA	3.6	1.9-4.0cm
FS	33	25-45%	RV		0.7-2.3cm (supine)
SWT		0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions:

LV wall thickness:

LV diastolic function:

SWMA.

LA size

G + 170

Aortic root dimension

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

RV = 19

Right side size and systolic function

Intracardiac masses and thrombi

Pericardium

IVC diameter and collapsibility.

Conclusion:

DL



Patient Name : الأستاذ / صباح وهيب يوسف مصطفى	Request No. : 13009827
Patient No. : 26312172100589	Request Date : 19/10/2024
Path No. : 3998/24	Verification Date : 31/10/2024
Sex : Female	Financial Category : اورام دار السلام - مبانره
Age : 61 Y	Department : Out Patient
Referreing Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left retro-areolar breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 17/10/2024

Gross Examination:

- Three grayish white tissue cores up to 2 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Not detected.
- * Perineural invasion: Not detected.
- * Angiolinvasion: Not detected.
- * Microcalcifications: Not detected.

Diagnosis:

Left retro-areolar breast mass., Tru-cut biopsy:

Invasive duct carcinoma., Grade II.

For further IHC (ER, PR, Her2, Ki67).

Signature
Dr.Salma Tark
Dr.Menna Nabil

تمت
د/مينا نabil

Verified by :





Patient Name	: الأستاذ / صباح وهيب يوسف مصطفى	Request No.	: 13009827
Patient No.	: 26312172100589	Request Date	: 19/10/2024
Path No.	: 3998/24	Verification Date	: 09/11/2024
Sex	: Female	Financial Category	: اورام دار السلام - ميلد
Age	: 61 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

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Verified by :





Patient Name الأستاذ / صباح وهيب يوسف مصطفى
Patient No. 26312 72100589
Path No. 3998/24
Sex Female
Age 61 Y
Referring Doc.

Request No. 13009827
Request Date 19/10/2024
Verification Date 09/11/2024
Financial Category اورام دار السلام - ميلد
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

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Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Dr.Salma Tarek
Dr.Esraa Magdy

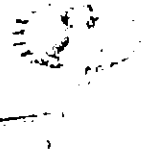
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Printing Date : 10/11/2024 10:38:43

Page 2 of 2

Printed By : .marwa mohamed yousse





Patient Name : الأستاذ / صباح وهيب يوسف مصطفى	Request No. : 13009827
Patient No. : 26312172100589	Request Date : 19/10/2024
Path No. : 3398/24	Verification Date : 31/10/2024
Sex : Female	Financial Category : اورام دار السلام - مبالغه
Age : 61 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left retro-areolar breast mass.

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- Three grayish white tissue cores up to 2 cm, totally submitted.

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- * Microcalcifications: Not detected.

Diagnosis:

Left retro-areolar breast mass., Tru-cut biopsy:

Invasive duct carcinoma. , Grade II.

For further IHC (ER, PR, Her2, Ki67).

Signature
Dr.Salma Tark
Dr.Menna Nabil

Verified by :





PP

وزارة الصحة والبيئة
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام

Echo report

Name:

سباغ وحبیب

Age:

٢٩٢٢
٢٤

Date: ٤/٩/ 2025

EF	62%	54-74%	PWT		0.6-1.1cm
LVEDD	4.9	3.5-5.7cm	AO		2.0-3.7cm
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SWT		0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions:

LV wall thickness:

LV diastolic function:

SWMA.

LA size

Aortic root dimension

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

RV = 19

Right side size and systolic function

Intracardiac masses and thrombi

Pericardium

IVC diameter and collapsibility.

Conclusion:

D/



Patient Name: Sabah Wahib Youssef Moustafa	Date : 11/11/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 61 Year(s)
Medical ID: 3933/2024	Pat. ID: 90734

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (15cm at Rt MCL) bright parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Surgically removed
- CBD is not dilated.

Spleen: -

Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal ; no abnormalities could be detected

Opinion:

Bright liver likely fatty

Cholecystectomy.

Colonic gaseous distention

Signature Dr . Nermien Ramadan

11/11/2024 2:51 PM



Patient Name: Sabah Wahib Youssef Moustafa	Date : 11/11/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 61 Year(s)
Medical ID: 3933/2024	Pat. ID: 90734

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Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected

Opinion:

Bright liver likely fatty
Cholecystectomy.
Colonic gaseous distention

Signature Dr . Nermien Ramadan

11/11/2024 2:51 PM



Patient Name	: الأستاذ / صباح وهيب يوسف مصطفى	Request No.	: 13009827
Patient No.	: 26312172100589	Request Date	: 19/10/2024
Path No.	: 3998/24	Verification Date	: 09/11/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 61 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Left breast retroareolar mass.

Nature of Specimen:

Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

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- PR: Moderate nuclear reaction in 10% of tumor cells (score 3/8).
- Her2: Complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 25% nuclear reaction of tumor cells.

Conclusion:

Left breast retroareolar mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 7/8).
- PR: Positive (score 3/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 25% of tumor cells.

Verified by :





Patient Name الأستاذ / صباح وهيب يوسف مصطفى
Patient No. 26312172100589
Path No. 3998/24
Sex Female
Age 61 Y
Referring Doc.

Request No. 13009827
Request Date 19/10/2024
Verification Date 09/11/2024
Financial Category اورام دار السلام - مبادره
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

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Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Dr. Salma Tarek
Dr. Esraa Magdy

Verified by :

Printing Date : 10/11/2024 10:38:43

Page 2 of 2

Printed By : .marwa mohamed yousse





Patient Name: Sabah Wahib Youssef Moustafa	Date : 15/10/2024
Exam : MAMMOGRAPHY BOTH	Age : 61 Year(s)
Medical ID: 26312172100589	Pat. ID: 90734

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Right breast palpable mass

Report:

- Left Breast:
 - Partially obscured high density spiculated mass is seen in the retro-areola region invading the nipple and ulcerating over the skin. It measures about 45x35mm. by U/S it is seen as a large irregular hypoechoic mass centered at 9 o'clock (extending into the surrounding tissue with intraductal extension reaching the nipple)
 - Another small suspicious mass is noted at 8O'clock measuring about 15x13mm
 - <3 Pathologically enlarged level I axillary LNs largest measures 15mm in TS diameter and 12mm cortical thickness.
 - No pathological enlarged infra or supraclavicular lymph nodes.
- Right Breast:
 - Scattered fibroglandular parenchyma (ACR b)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Left locally advanced breast malignant mass with ipsilateral axillary lymphadenopathy (BIRADS 5) for histopathological examination.
- Normal sonomammography of right breast (BIRADS 1)

Best regards,

Dr Eman Ebeid

Dr. Amira H Radwan MD,
Breast Imaging consultant,

NCI, Cairo University

15/10/2024 3:33 PM



Patient Name: Sabah Wahib Youssef Moustafa	Date : 11/11/2024
Exam : C.T ABDOMEN & PELVIS	Age : 61 Year(s)
Medical ID: 3933/2024	Pat. ID: 90734

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS

Findings:

I Chest:-

- Left breast retroareolar heterogenous soft tissue density mass with spiculated outlines is seen inseparable from nipple measures about 5.5x4.5x4cm with diffuse overlying skin thickening. It is seen surrounded by small satellites.
- Left axillary solitary pathologically enlarged lymph node measures 2.7x2.4 cm, **Otherwise**, bilateral prominent other axillary level I lymphnodes with preserved hila are also noted.
- Diffuse pulmonary mosaic attenuation pattern is seen showing ground glass mottling intermingled with areas of low parenchymal; attenuation of air trapping with bilateral fine atelectatic bands, **Otherwise** clear both lung fields with no definite suspicious nodules, masses or cavitary lesions.
- Small subcentimetric mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild cardiomegaly, No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.



Patient Name: Sabah Wahib Youssef Moustafa	Date : 11/11/2024
Exam : C.T ABDOMEN & PELVIS	Age : 61 Year(s)
Medical ID: 3933/2024	Pat. ID: 90734

***Scanned skeleton:** no definite worrisome lytic or sclerotic focal lesions.

Opinion:

- Left breast malignant looking mass with surrounding small satellites, as described for SM correlation.
- Left axillary level I solitary pathological lymphnode
- No hepatic or osseous depositions

Dr/Reem Elwaly

Dr. Walid Mohamed, M.D.



Patient Name : الأستاذ / صباح وهيب يوسف مصطفى	Request No. : 13009827
Patient No. : 26312172100589	Request Date : 19/10/2024
Path No. : 3998/24	Verification Date : 31/10/2024
Sex : Female	Financial Category : اورام دار السلام - مبادئه
Age : 61 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left retro-areolar breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 17/10/2024

Gross Examination:

- Three grayish white tissue cores up to 2 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Not detected.
- * Perineural invasion: Not detected.
- * Angioinvasion: Not detected.
- * Microcalcifications: Not detected.

Diagnosis:

Left retro-areolar breast mass., Tru-cut biopsy:

Invasive duct carcinoma., Grade II.

For further IHC (ER, PR, Her2, Ki67).

Signature
Dr.Salma Tark
Dr.Menna Nabil

Verified by :





Patient Name	: الأستاذ / صباح وهيب يوسف مصطفى	Request No.	: 13009827
Patient No.	: 26312172100589	Request Date	: 19/10/2024
Path No.	: 3998/24	Verification Date	: 09/11/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبلّره
Age	: 61 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

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Left breast re'troareolar mass.

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Left breast retroareolar mass, Tru-cut biopsy:

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Verified by :





Patient Name الأستاذة / صباح وهيب يوسف مصطفى
Patient No. 26312172100589
Path No. 3998/24
Sex Female
Age 61 Y
Referring Doc.

Request No. 13009827
Request Date 19/10/2024
Verification Date 09/11/2024
Financial Category اورام دار السلام - مبادره
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, Intense < 10% of tumor cells.

Dr. Salma Tarek
Dr. Esraa Magdy

Verified by :

Printing Date : 25/12/2024 11:37:34

Page 2 of 2

Printed By : Mrs. Mayada Hegazy





Patient Name: Sabah Wahib Youssef Moustafa	Date : 11/11/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 61 Year(s)
Medical ID: 3933/2024	Pat. ID: 90734

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (15cm at Rt MCL) bright parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Surgically removed
- CBD is not dilated.

Spleen: -

Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected

Opinion:

Bright liver likely fatty
Cholecystectomy
Colonic gaseous distention

Signature Dr . Nermien Ramadan

11/11/2024 2:51 PM



Patient Name الأستاذ / صباح وهيب يوسف مصطفى
Patient No. 26312172100589
Path No. 3998/24
Sex Female
Age 61 Y
Referring Doc.

Request No. 13009827
Request Date 19/10/2024
Verification Date 09/11/2024
Financial Category اورام دار السلام - مبادره
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCC-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Dr.Salma Tarek
Dr.Esraa Magdy

Verified by :

Printing Date : 10/11/2024 10:38:43

Page 2 of 2

Printed By : .marwa mohamed youss



Patient Name : الأستاذ / صباح وهيب يوسف مصطفى

Request No. : 13005889

Patient No. : 26312172100589

Request Date : 18/05/2025

Path No. : 1817/25

Verification Date : 03/06/2025

Sex : Female

Financial Category : اورام دار السلام - مائترة

Age : 61 Y

Department : Out Patient

Referring Doc :

Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer (pcst NACT) .

Nature of the specimen: Left MRM

Gross Examination:

• Procedure: Left MRM

measured 24x20x7 cm with covering skin ellipse measured 22x20cm & attached axilla 9x6 cm .

• Tumor extent : A greyish white tumour bed site

• Size: 4x3x3 cm .

• Skin involvement: Nipple & areola were unremarkable grossly .

• Distance from margins: 2 cm away from deep margin

• Lymph nodes number and size: 14 L.Ns up to 2x1 cm .

Microscopic examination:

• Histological type: No residual viable tumor tissue detected

• Ductal carcinoma in situ: No

• Skin involvement: Free nipple and areola

• Lymph node involvement: Free L.Ns (0/14)

• Resection Margins: Free deep margin

• Additional pathological findings: Positive microcalcification

• Histologic response to therapy: Pathologic Complete response

Diagnosis:

Left MRM (post NACT) :

No residual viable tumor tissue detected.

• Free deep margin

• Free nipple and areola

• Free L.Ns (0/14)

• Pathologic Complete Response

• Stage : ypT0 , ypN0

Verified by :



Patient Name : الأستاذة / صباح و هيب يوسف مصطفى

Patient No. : 26312172100589

Path No. : 1817/25

Sex : Female

Age : 61 Y

Referreing Doc :

Request No. : 13005889

Request Date : 18/05/2025

Verification Date : 03/06/2025

Financial Category : أورام دار السلام - مبانره

Department : Out Patient

Registered by : Mrs.Mayada Hegazy

Pathology Report

Signature

Dr. Walaa Ghoenim

Dr.Heba Hassan

Verified by :





وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

أوامر الطبيب Doctor's Orders

.....إسم المريض ثلاثي: د. محمد أحمد
.....رقم الملف: ٣٩٢٢
.....C-٤٦

..... رقم الملف:



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أودام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders **اوامر الطبيب**

التفخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		حالة المريض كما يلي: (ب) (ج) (د) (هـ) تحت إشراف د. محمد مستشفى	
	٥/١٧	د. محمد د. أحمد ٥/١٧	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

ECHO REPORT

Name: صباح وهيب يوسف

Date: 22 / 7 / 2025

EF	55-60%	55-75%	PWT	0.7	0.6-1.1cm
LVEDD	5.0	3.5-5.8cm	AO.R	2.5	2-3.7cm
LVEDS		2-4cm	LA	3.4	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

* The patient has frequent Premature beats

LV dimensions ———— 0ml

LV wall thickness ———— 0ml

LV diastolic function D-D grade

SWMA ———— SW 1A

LA and Ao root dimensions ———— 0ml

Mitral Valve Mild TR

Aortic valve ———— 0ml

Tricuspid valve Mild TR

Pulmonary valve ———— 0ml

Right side and systolic function ———— 0ml

Intracardiac masses and effusion ———— 0

The patient is Tachycardic for Control of HR & reassessment
& ECG correlate

Dr / Mina Adel

Signature

اسم المريض ثلاثي:
رقم الملف:



MINISTRY OF HEALTH
937 927815
ساح وادي موهب
الرياض
25312 (72100589)
2024

أه
مست

Oncology Follow up Sheet

صحيفة متابعة مريض أورام (☐ أطفال / ☐ كبار)

اسم المريض:	
الرقم الطبي:	رقم الملف:
المعاملة المالية:	الرقم القومي:
الطبيب المعالج:	
العنوان الدائم:	
رقم الهاتف:	الجنسية:
النوع:	السن:
تاريخ الميلاد:	تاريخ أول زيارة:
كود الأشعة:	

.....: اسم المريض ثلاثي:
.....: رقم الملف:



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PHYSICIAN ASSESSMENT SHEET

I. PERSONAL HISTORY:-

Occupation:
Marital Status : ☐ Married ☐ Single ☐ Widow ☐ Divorced
Special Habits : ☐ Smoking ☐ Coffee ☐ Alcohol ☐ Others
Allergy: ☐ No ☐ Yes - Food: Medications: Others: ☐ Unknown

II. HISTORY:

1.Past History: - ☐ DM ☐ HTN ☐ CLD ☐ IHD ☐ CRD ☐ CVS

☐ Others, specify:

☐ Previous Operations, specify:

☐ Previous Admission to Hospital, ☐ No ☐ Yes Comment

2.Family History:

.....
.....
.....

- Menstrual history : Age at Menarche : / Y Ferquency : / Days .
- Regular : No / Yes Duration : / Days Pain: No / Yes Discharge: No /
- Menopause: No / Yes Duration : / Y
- History of location:
- History of Miscarriage still birth : No / Yes (Times)
- Children: No. : Sex: Age:

3.Psychological History (Including Emotional/ Behavior/ Social Factors)

.....
.....
.....
.....
.....
.....

4.Current Medication:

Drug	Strength	Dosage/ Frequency	ROUTE

5.Date of First Diagnosis of Cancer

/ / 20

6.Date of First Presentation in Our Unit

/ / 20

7.Present History:

A. The Condition Started by:

.....
.....
.....

اسم المريض ثلاثي: ج. ج. ج. ج.
رقم الملف: ٤٤/٢٩٢٢



وزارة الصحة و السكان
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مستشفى اورام دار السلام "هرمل"

for NACTCH

Adj. Bw: 67.5 Kg
Age: 61
wt: 97
Ht: 155
s.A: 2

III. INVESTIGATIONS :

Laboratory Results:

Date: / /20	T BIL:	Creat:
HB:	D Bill:	Urea:
Plt:	AST:	ALP:
WBC: Neut:	ALT:	Ca: K:

Date of Echo: / /20

LVEF= %

> Tumor Markers:

٧١

مستوى ألفا فيتوبروتين مرتفع جداً

> Other test:

تكرار (١٢) م ٢٠

مستوى بروتين سي المناعي مرتفع جداً

مستوى بروتين سي المناعي مرتفع جداً

مستوى بروتين سي المناعي مرتفع جداً

مستوى بروتين سي المناعي مرتفع جداً

> Pathology and immunohistochemistry

١٢٠

خارجي
١٢/١٦/٢٠٢٤

[164mg]
١٢/١٦/٢٠٢٤

١٢/١٦
١٢/١٦

.....إسم المريض ثلاثي:
.....رقم الملف:



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➤ Other Investigations (Imaging, Biopsy, Endoscopy, Bone marrow, Biochemistry)

IV. PREVIOUS CHEMOTHERAPY:

V. PREVIOUS RADIOTHERAPY:

VI. PREVIOUS MANAGEMENT OF OTHER DISEASES:

VII. PHYSICAL EXAMINATION:

1. General Appearance: ☐ Quiet ☐ Distressed ☐ Irritable ☐ Other
2. Color: ☐ Normal ☐ Cyanosed ☐ Jaundiced ☐ Pale
3. Pain: ☐ No ☐ Yes Specify :
4. Hydration: ☐ Normal ☐ Dehydrated

General Examination

Vital Signs:	Pulse	Temp	BP	RR	Weight	Height	Orientation	Conscious Level

.....إسم المريض ثلاثي:
.....رقم الملف:



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• PERFORMANCE STATUS AT TIME OF PRESENTATION

• Head and Neck

• Chest and Heart

• Abdomen and Pelvis

• Limbs

• Regional Lymph nodes

➤ Site:..... Single: O..... Multiple O.....
➤ Fixed: O..... Mobile: O..... Painless: O..... Painful: O.....

• Non Regional Lymph nodes

➤ Site

Neurological findings.....

Genitalia.....

اسم المريض ثلاثي:
رقم الملف:



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VIII. DIAGNOSIS:

• Provisional Diagnosis:

• Final Diagnosis:

• ICD-O-3:

Doctor Signature: Date: / /20 Times:.....

IX. MANAGEMENT PLAN:

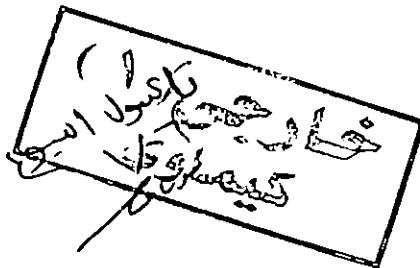
Palliation / Radical / Unfit / Radiotherapy / Surgery / Chemotherapy

Handwritten notes and signatures in Arabic, including a large signature and various medical notations.

Decision by: prof. DR.

Date : / /

Time :



إسم المريض ثلاثي:
رقم الملف:



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Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☐ Treatment

☐ Follow-up

Clinical findings:

.....
.....
.....
.....
.....
.....
.....
.....
.....
.....

	Doctor Order	الوقت
EF; % BP; TEMP; HR; HB; WBC: PLT: UREA: CREAT: ALT: AST: T.BIL: D.BIL:	ملاحظة: لا توجد مشاكل في الفحوصات د. محمد عبد الله	11/11

Doctor Signature: Date: / / 20 Time:

اسم المريض ثلاثي: جيباج و. هادي
رقم الملف: ٢٠٢٢/١١١١١١١١



وزارة الصحة و السكان
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مستشفى اورام دار السلام " هرمل "

Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☐ Treatment

☐ Follow-up

Clinical findings:

المرضى في حالة جيدة
لا توجد أعراض جديدة
الوزن ٦٠ كجم
الضغط ١٢٠/٨٠ مم زئبق
سكر الدم ١٠٠ ملغم/دلي

الوقت	Doctor Order
EF; BP; TEMP; HR; HB; WBC; PLT; UREA; CREAT; ALT; AST; T.BIL; D.BIL;	المرضى في حالة جيدة لا توجد أعراض جديدة الوزن ٦٠ كجم الضغط ١٢٠/٨٠ مم زئبق سكر الدم ١٠٠ ملغم/دلي labs. → ٥٠٤ (٦/١) → ٥٠٤ seru → ١٠١ N.B (P) Anti-HER2 Approval by HoHP W5 TC (AUC=2) CLPh/ ٥٠٤ (13/1/2025)

Doctor Signature: Date: 1 / 20 Time:

Still at: - Hu 2 not approved
صباح و هيب
< ٤١٢٩٣٣

Dr R
Lk 6

gxl 140 mm, J
Cathol AIC 2
J. H. S.

ridge in
Benton 300
W. H. S.
100



mtl
Conte
Dr

W. H. S.
Conte
J. H. S.
100

pt Res neuropathy
risk not ent.
100

Lab 611
Dr. Cr 1.1
ord ST m/m
TC 184
J. H. S.
2011

اسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام " هرمل "

Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☒ Treatment

☐ Follow-up

* C/O: G. & Diarrhea.
- Burning Micturition

Clinical findings:

* Base scar 11.1.2025: Free

* Full labs: OK

* قران Phary → still pending approval

* Subjective improvement in Lt. Breast skin changes
↓ in size of Lt. Breast ulcer

	Doctor Order	الوقت
EF; %	* O/E:	
BP;	Rt. Lt. → small soft Lt. axill LN ~ 2.5cm.	
TEMP;	↓ in Lt. Breast skin changes.	
HR;	Lt. Breast Mass nearly stationary ~ 4.5 x 4cm.	
HB;	↓ in size of Lt. Breast ulcer, now ~ 1.5 x 2.5cm.	
WBC;		
PLT;		
UREA;	* For: Taxol 1Carb. Today.	
CREAT;	[W7] 140mg → AUC [2]	
ALT;	[W8] Taxol 140mg 1Carb. AUC [2] on 3/2/2025	
AST;	[W9] Taxol 140mg 1Carb. AUC [2] on 20/2/2025	
T.BIL;	Gastrazole 40mg Tab. m.e daily for 20 day.	
D.BIL;	Motilium Tab. twice daily for 5 day.	
	Hidrasec Tab. twice daily for 5 day.	
	Cipro 500mg Tab.	

Doctor Signature: Date: 27/1/2025 Time:

F-SMC-MR-ONC-PHY-001
Issue No/Review No: 01/00
Issue Date: 1/7/2024

Page 10 of 64

Overkals
due to shortage

سے (در)

5:50 15/11

0 10110
فيسبوك
05 9 علاء عبد

[illegible]

omit comb c
due to
slow Azox

خارجی (1)

وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "

Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

Treatment

☐ Follow-up

Clinical findings:

675 on Hwy 1. Tully/line
 (A) on the yard on Park

Doctor Order		الوقت
EF; %	For	P.N. Cat
BP;		
TEMP;	W ₁₀ T _{axal} 140 / Carb A _{u2}	
HR;	W ₁₁ Physio 60 S-L T _{axal} 140 / Carb A _{u2}	
HB;		
WBC:	W ₁₂ T _{axal} 140 / Carb A _{u2}	
PLT:		
UREA:		
CREAT:	W ₁₂ TC [189 ng]	
ALT:		
AST:		
T.BIL:		
D.BIL:		

Doctor Signature: _____ Date: 7 / 2 / 2025 Time: 11:00 AM

Wiel $P_1 \vee P_2 \wedge P_3 \vee P_4 \wedge P_5 \wedge P_6$

120 ~~118~~ 09-12

(8/17) AuCl_2 in H_2SO_4

W13TC
tHPD
2010
4/10

NSI. $\sqrt{600/600}$ 9

~~یوحنا بابا~~

لو زلالہ

جواب -

فرض کنید

— حضرت

Handwritten signature
P/W

ω_{14}

W15

Handwritten note: **مجلس** (Majlis) with a checkmark and the number **٢٤٤٤**.

7W14TC (163mg)

9.00 14/18

2. به استحضار

17/11/2018

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مستشفى أورام دار السلام " هرمل "

Follow up // Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☐ Treatment

☐ Follow-up

Clinical findings:

۱۱۱
 (۱۱ + ۲) (۱۱ + ۲) (۱۱ + ۲)
 ۱۱۱ (۱۱ + ۲) (۱۱ + ۲) (۱۱ + ۲)
 ۱۱۱ (۱۱ + ۲) (۱۱ + ۲) (۱۱ + ۲)

Doctor Order		الوقت
EF;	%	
BP;		
TEMP;		
HR;		
HB;		
WBC;		
PLT;		
UREA;		
CREAT;		
ALT;		
AST;		
T.BIL;		
D.BIL;		

Doctor Signature: **Date:** / / 20 **Time:**

К. В. Ефремов

۱۰۲

۱۴۰۲

AUC2 ≈ 0.5

سید علی محمد علی
محمد علی محمد علی

فتو فی سیر

Libre C'

Report clinical
improvement

CBC $\rightarrow \checkmark$
 GNS $\rightarrow \checkmark$
 Sec or $\rightarrow 1, 2$ } (24/3)

C_3H_8Sg
 W_2T
 EIV

Labson 1414
srer 1.4

W17

C₃ phasgo
+ W₁₆ TC (AUC=2)
CLPH/12 JG
(7/14/2025)

25) உருவத்திலும், உருவத்திலும்

Q. 17

120

Auc7 mil, B

~~Cell postpone~~

W 7. TC (139)

7424

ClpH

سورخان: ۱۰۰
جرا: ۱۰۰

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۱۲/۱۰۰۰ ۰ ۱۲/۱۰۰۰ ۰ ۱۲/۱۰۰۰

اسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "

Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☐ Treatment

☐ Follow-up

Clinical findings:

Wt on 14/4 was postponed
due to tachycardia
acc to physician recommendation
14/4

	Doctor Order	اتوقت
EF; %		
BP;		
TEMP;		
HR;		
HB;		
WBC:		
PLT:		
UREA:		
CREAT:		
ALT:		
AST:		
T.BIL:		
D.BIL:		

Doctor Signature: Date: / / 20 Time:

۷۴ - ۷۷

رہنمائی + ایک، ایک، ایک

W17TC (145ms)
2014/2025
دستور

تاکوں (۱۰، ۲۸، ۱۰)

کاربرینہ A222

مذہب - ۷۷، ۷۸، ۷۹

حائزین زہر، ادب

۷۷، ۷۸، ۷۹

بازار، رستوران، کافہ

۷۷، ۷۸، ۷۹

W18TC (AOC=2)

+ C4 phesgo
CLPh/ ۷۸، ۷۹
(28/4/2025)

New EF
Before Next cycle

صفحه ۱۵۰
۷۷، ۷۸، ۷۹

۷۷، ۷۸، ۷۹
C.C. ۷۷، ۷۸، ۷۹

EF 62% (14/5/2025)

C5 phesgo
۷۷، ۷۸، ۷۹
C.C. ۷۷، ۷۸، ۷۹

W18TC + Carbo
۷۷، ۷۸، ۷۹
C.C. ۷۷، ۷۸، ۷۹

C5 فیس پیجو - ۷۷، ۷۸، ۷۹
بازار، رستوران، کافہ

۷۷، ۷۸، ۷۹
C.C. ۷۷، ۷۸، ۷۹

جابتق ۷۷، ۷۸، ۷۹ C.C. ۷۷، ۷۸، ۷۹

۷۷، ۷۸، ۷۹

اسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام "اهرمل"

Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☐ Treatment

☐ Follow-up

Clinical findings:

- Dexa + Avil
C6 phesgo 600/600 S.C
- Finarm
- Calcimate } 21 day
- Bone ma.
J.P. 22-7-225
C6 phesgo
C1 Pharmacy
Doctor Order
C1 Col VEC
ECHO 22-7-225
14-5-225
EF 62%

	الوقت
EF; %	
BP;	
TEMP;	
HR;	
HB;	
WBC;	
PLT;	
UREA;	
CREAT;	
ALT;	
AST;	
T.BIL;	
D.BIL;	

C7 : phesgo 600/600 S.C
sulfax
- Aramidex 1
- Calcema
- Bone
- meenyl tech
Echo (22-7) 55-60%
Repeat after this cycle
C7 phesgo
Ahmed Mohamed
12-8
For 1 month.
اسماء الطوبى
٢٥/١/٢٢

Doctor Signature: Date: / / 20 Time: